



Oral Cavity & Salivary Glands

Pathology GIT Block · MEDucation by Mattin Majid · Practice MCQs

HOW TO USE THIS SET

20 questions, mixed difficulty, 8 case-based. Attempt every question before checking the answer key on the last page.

4 Easy

10 Medium

6 Hard

1.

EASY

Inflammation of the tongue is termed:

- A. Cheilitis
- B. Glossitis
- C. Gingivitis
- D. Stomatitis
- E. Sialadenitis

2.

EASY

Approximately what proportion of oral cavity cancers are squamous cell carcinoma?

- A. 50%
- B. 65%
- C. 80%
- D. 95%
- E. 99%

3.

EASY

The commonest lesion of the minor salivary glands is:

- A. Warthin tumour
- B. Mucocele
- C. Oncocytoma
- D. Adenoid cystic carcinoma
- E. Sialadenitis

4.

EASY

Salivary gland neoplasms make up approximately what proportion of all head and neck neoplasms?

- A. 2%
- B. 10%
- C. 25%
- D. 40%
- E. 50%

5.

MEDIUM

CASE

A 61-year-old pipe smoker has a red, velvety, slightly depressed patch on the floor of the mouth that does not wipe off with gauze. An identical-sized white plaque elsewhere in his mouth also fails to wipe off.

Compared with the white plaque, the red lesion carries a malignant transformation risk that is:

- A. Lower
- B. Roughly equal
- C. Substantially higher
- D. Zero, since red lesions are never premalignant
- E. Impossible to estimate without biopsy of both

6.

MEDIUM

A patch is provisionally called leukoplakia. Which single finding on examination would force you to abandon that label?

- A. The patch is on the buccal mucosa
- B. The patient uses smokeless tobacco
- C. The patch wipes away completely with gauze
- D. Histology later shows dysplasia
- E. The patch has been present for over a year

7.

MEDIUM

CASE

A pregnant woman develops a rapidly enlarging, richly vascular, ulcerated nodule on her gingiva that bleeds easily on contact.

What is the most likely diagnosis?

- A. Fibroma
- B. Mucocele
- C. Pyogenic granuloma
- D. Mucoepidermoid carcinoma
- E. Pleomorphic adenoma

8.

MEDIUM

CASE

A 6-year-old develops grouped 1–3 mm vesicles around the lip margin that crust over and resolve within 8 days. Biopsy of a similar earlier lesion showed glassy intranuclear inclusions in squamous cells.

This clinical course is most consistent with which oral lesion, and NOT with the differential most likely to be confused with it?

- A. Recurrent herpetic stomatitis, not aphthous ulcer
- B. Aphthous ulcer, not herpetic stomatitis
- C. Pyogenic granuloma, not fibroma
- D. Mucocele, not sialadenitis
- E. Candidiasis, not leukoplakia

9.

MEDIUM

A patient has recurrent, painful oral ulcers with a fibrinopurulent base and an erythematous halo, each healing within 7–10 days. Given the associations of this lesion, which underlying condition is most worth asking about?

- A. Hepatitis C infection
- B. Coeliac disease
- C. Chronic kidney disease
- D. Peripheral vascular disease
- E. Iron deficiency alone, with no systemic association

10.

MEDIUM

CASE

A 58-year-old man who both smokes heavily and drinks alcohol daily is told that his combined risk of oral SCC is far greater than either habit would predict alone.

This pattern is best explained by:

- A. Tobacco and HPV acting through the same molecular pathway
- B. An additive but independent effect of the two exposures
- C. A synergistic interaction between alcohol and tobacco
- D. Alcohol acting only as a marker of tobacco exposure, with no independent effect
- E. HPV infection being required for either exposure to cause harm

11.

MEDIUM

A tumour of similar size is found in the sublingual gland of one patient and the parotid gland of another. Compared with the parotid tumour, the sublingual tumour is:

- A. More likely to be malignant
- B. Less likely to be malignant
- C. Equally likely to be malignant
- D. Almost certainly a pleomorphic adenoma
- E. Not able to be compared without a biopsy of both

12.

MEDIUM

CASE

Histology of a slow-growing, painless parotid mass shows ductal (epithelial) and myoepithelial cells set in a myxoid, chondroid matrix.

This tissue combination is most characteristic of:

- A. Mucoepidermoid carcinoma
- B. Warthin tumour
- C. Pleomorphic adenoma
- D. Adenoid cystic carcinoma
- E. Acinic cell carcinoma

13.

MEDIUM

A 55-year-old man has a biopsy-proven oral squamous cell carcinoma of the lateral tongue. Which site is checked first for regional lymphatic spread?

- A. Mediastinal nodes
- B. Cervical lymph nodes
- C. Coeliac nodes
- D. Supraclavicular nodes
- E. Inguinal nodes

14.

MEDIUM

HPV genotypes 16 and 18 are implicated in oral SCC. What proportion of cases carry this association?

- A. Under 5%
- B. About 10%
- C. 30-50%
- D. Over 90%
- E. Essentially all cases

15.

HARD

CASE

A patient's oral biopsy report reads: "increased keratinisation, no dysplasia identified."

How should this leukoplakia be regarded?

- A. Benign and requiring no further follow-up, since dysplasia is absent
- B. Reclassified as candidiasis, since dysplasia is absent
- C. Still precancerous, since leukoplakia is precancerous until histology proves otherwise across its full spectrum
- D. Diagnostic of invasive carcinoma
- E. Automatically reclassified as erythroplakia given the degree of keratinisation

16.

HARD

CASE

A patient underwent excision of a parotid mass ten years ago that was reported as pleomorphic adenoma, with a note that margins were close. He now returns with a firm, rapidly enlarging, painful mass at the same site, with new facial weakness.

This clinical course is most consistent with:

- A. Simple recurrence of the original pleomorphic adenoma
- B. Malignant transformation — carcinoma ex pleomorphic adenoma
- C. A new, unrelated Warthin tumour
- D. Reactive lymphoid hyperplasia from the original surgery
- E. Sialadenitis secondary to duct obstruction

17.

HARD

Which of the following is NOT among the recognised causes of sialadenitis?

- A. Mumps virus
- B. Staphylococcal infection following duct obstruction
- C. A salivary duct stone
- D. Sjögren syndrome
- E. HPV infection

18.

HARD

CASE

Two salivary gland tumours are compared: Tumour A shows squamous, mucus-secreting, and intermediate cells. Tumour B shows ductal and myoepithelial cells with chondroid stroma.

Which statement correctly ranks these two tumours?

- A. Tumour A is the commonest benign tumour; Tumour B is the commonest malignant tumour
- B. Tumour A is the commonest malignant tumour; Tumour B is the commonest benign tumour
- C. Both are equally common and both are benign
- D. Tumour A is more common overall but Tumour B carries the higher recurrence risk
- E. Tumour B is malignant and Tumour A is benign

19.

HARD

CASE

A 67-year-old heavy smoker and drinker has an indurated, non-healing oral ulcer of 8 weeks' duration. He asks whether a course of antifungal treatment and review in three months would be reasonable, since a similar patch resolved with treatment before.

What is the most appropriate response?

- A. Agree — an antifungal trial is reasonable first-line management given his prior history
- B. Agree, provided he is reviewed sooner than three months
- C. Disagree — a non-healing ulcer of this duration in a high-risk patient warrants biopsy, not empirical treatment or reassurance
- D. Disagree only because three months is too short an interval
- E. Agree, since oral SCC rarely presents as a non-healing ulcer

20.

HARD

Which statement about salivary gland tumour distribution is TRUE?

- A. The parotid contributes the fewest tumours of the three major sites but has the highest proportion malignant
- B. The submandibular gland has a higher proportion of malignant tumours than the parotid but a lower proportion than the sublingual/minor glands
- C. The sublingual and minor glands contribute the fewest tumours overall but the highest proportion are malignant
- D. The parotid gland has the highest proportion of malignant tumours of the three sites
- E. Malignant potential is unrelated to gland size

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	B	Glossitis is inflammation of the tongue (glossa = tongue).
2	D	95% of oral cavity cancers are SCC.
3	B	Mucocele is the commonest lesion of the minor salivary glands.
4	A	Salivary gland neoplasms are only about 2% of head and neck neoplasms.
5	C	Erythroplakia carries >50% malignant transformation risk vs 3–25% for leukoplakia — substantially higher despite similar appearance in size.
6	C	Leukoplakia is defined as a white patch that cannot be scraped off; a patch that wipes away is candidiasis, not leukoplakia — dysplasia on histology does not exclude the label, it confirms concern within it.
7	C	Pyogenic granuloma classically occurs on the gingiva in pregnancy — the "pregnancy tumour".
8	A	Grouped vesicles with glassy intranuclear inclusions and a short course are recurrent herpetic stomatitis (HSV-1); aphthous ulcers lack vesicles and inclusions.
9	B	Aphthous ulcers are linked to coeliac disease, IBD, and Behçet — coeliac disease is the systemic association worth screening for.
10	C	Alcohol and tobacco interact synergistically to raise oral SCC risk far beyond an additive effect; HPV acts through a separate, largely independent pathway.
11	A	The smaller the gland, the higher the proportion of its tumours that are malignant — sublingual/minor glands are only ~40% benign vs 80% for parotid.
12	C	Ductal + myoepithelial cells in a myxoid/chondroid matrix defines pleomorphic adenoma; mucoepidermoid carcinoma shows squamous, mucous and intermediate cells instead.
13	B	Cervical lymph nodes are the most common regional site of spread for oral SCC.
14	C	HPV 16/18 are found in 30–50% of oral SCC cases, marking a distinct subset.
15	C	Every leukoplakia remains precancerous until proven otherwise by histology — the label spans the full range from increased keratinisation without dysplasia through to invasive SCC.
16	B	Recurrence risk is expected after close-margin excision, but a new rapidly growing, painful mass with nerve involvement years later signals malignant transformation — carcinoma ex pleomorphic adenoma.
17	E	HPV is not among the described causes of sialadenitis (trauma, viral/mumps, bacterial, autoimmune/Sjögren, and obstruction by stone or dehydration are).
18	A	Tumour A (squamous + mucous + intermediate cells) is mucoepidermoid carcinoma, the commonest malignant salivary tumour; Tumour B (ductal + myoepithelial + chondroid) is pleomorphic adenoma, the commonest benign tumour.
19	C	A non-healing oral ulcer of this duration in a high-risk patient needs biopsy — empirical treatment or a "wait and review" plan risks missing SCC.
20	C	Sublingual/minor glands contribute the smallest share of tumours (5%) but the highest proportion malignant (~60%); parotid contributes the most tumours (80%) but the lowest proportion malignant (~20%).

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Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.